Indiana Health Information Exchange

June 16, 2025

Centers for Medicare & Medicaid Services
P.O. Box 8013
Baltimore, MD 21244-8013

Re: CMS-0042-NC.

Dear Assistant Secretary:

We appreciate the opportunity to respond to the Request for Information (RFI) on the Health Technology Ecosystem and to share our feedback as one of the nation's first and longest operating health information exchange (HIE) organizations.

The Indiana Health Information Exchange (IHIE) is a nonprofit corporation serving the state of Indiana to facilitate and support secure, interoperable data exchange. IHIE was founded in 2004 and has become one of the largest HIEs in the U.S. IHIE connects over 38 health systems, 19,000 practices, and 54,500 providers with public health agencies, providers, and health plans with data covering more than 20 million patients.

Below, we offer our response on certain areas of focus involving the role of HIEs, interoperability, and TEFCA.

On behalf of IHIE and other HIEs, we appreciate your consideration of our responses and welcome the opportunity to collaborate with CMS and ASTP/ONC.

Sincerely,

John Kansky
Chief Executive Officer, IHIE

846 N. Senate Avenue, Suite 110 | Indianapolis, IN 46202 | Ph: 317.644.1750 | F: 317.222-3800 | www.ihie.org

Indiana Health Information Exchange

The Role of HIEs in Support of Health Information Access, Data Exchange, and Interoperability
(PC-11, PR-3, PR-7, PA-5)

For various reasons, most HIEs do not enable direct patient access to information. Instead, HIEs support provider and payor efforts to engage with patients by using their data assets to populate and enhance patient portals offered by providers, payors, or digital health application vendors and facilitate the exchange of health data to promote better outcomes, quality of care, and efficiency. The availability and exchange of EHR data is necessary not only for safe, high-quality patient care, but for many purposes beyond point-of-care decision-making. Among these are quality measurement, costs analysis and control, and care coordination. Furthermore, the data needed for these programs does not reside solely within provider EHRs. Claims data, commercial labs, and imaging centers maintain data that is essential to furthering the quality, safety, and efficiency of health care. No single payor or provider organization has all the necessary data to measure the quality of healthcare; what an individual provider organization or payor might identify as a gap in care may be, in actuality, just a lack of knowledge (data) that care occurred. Statewide HIEs and health data utilities (HDUs) aggregate data from across the provider continuum as well as from commercial and government payors. The resulting HIE/HDU data assets are valuable for quality measurement and can improve the accuracy of quality measurement and reduce the overall burden of calculating and reporting such measures.

While the value, availability, and accuracy of data shared by HIEs varies, the nation's leading HIEs/HDUs maintain quality data from many participating sources across an entire state. Leading HIE/HDU states are well-known and their models can be studied and scaled. These include Arizona, Colorado, Indiana, Nebraska, and Maryland. One of the reasons that robust, statewide HIEs do not exist in more states is that past federal policies have been focused on EHR-vendor networks and TEFCA. While federal policies will be needed to guide and achieve true national interoperability, state-based, non-profit or state-run HIEs and HDUs should be the cornerstone of interoperability for the following reasons:

1) Trust -- Recent disputes between for-profit vendors exposed a lack of trust in national, vendor-controlled networks. State-based HIEs and HDUs have been recognized as having a high degree of trust with their participating organizations, in part due to the multi-stakeholder governance of the HIE/HDU and the relative "localness" of relationships.
2) Lack of profit motive -- While health information is subject to federal protections, it is also increasingly economically valuable. There will always be a minority of for-profit companies that will be motivated to monetize the value of personal information.
3) Neutrality -- For decades competitive interests between HIT vendors and within stakeholders in the healthcare system have been obstacles to the essential exchange of patients' information. Not-for-profit, state-based HIEs and HDUs gain diverse participation in part because of their non-competitive position in the market.

Statewide HIEs and HDUs work cooperatively with providers and payors across the continuum to aggregate, clean, and normalize the healthcare data of citizens of their state. This lessens the burden and costs incurred by providers to make high-quality data available for its various purposes, including quality measurement and improvement. CMS and ASTP ONC could help providers, payors, state governments, and the digital product ecosystem by reframing policies and guidelines to support existing HDUs and to accelerate the maturation of HIEs into HDUs that connect with a wider range of data sources and provide more in depth insights and analysis with the reporting and access offerings available to stakeholders.

We propose to CMS and ONC that:

- A nationwide strategy to give patients better access to their data should include HIEs as data assets to populate, supplement, and enhance patient portals offered by providers, payors, or digital health application vendors.
- A nationwide strategy to enhance quality measurements should include HIEs as the aggregator of relevant data from a variety of sources.
- A nationwide strategy to promote interoperability should include increased support to existing exchange infrastructure to support and accelerate the maturation of HIEs into HDUs.

Adoption of the Trusted Exchange Framework and Common Agreement (TEFCA)
(PR-6, PR-11, TD-6)

TEFCA is not yet capable of helping to advance provider access to health information; it is still nascent. While Epic is choosing to migrate its customers to TEFCA, those connections are just replacing interoperability that already existed via Carequality and do not represent new participants in national data exchange. Participation by organizations other than Epic customers is still very low. Transaction volume under TEFCA is primarily "patient discovery" -- not the actual exchange of useful clinical information. Public health transaction volumes are extremely low and queries that are sent often receive no responses.

While federal policies are necessary to incentivize interoperability and prohibit malevolent use of protected health information, those policies should be laser-focused on known obstacles to increased national interoperability. TEFCA is a heavy and overly prescriptive implementation of federal policy. Prior to TEFCA, an imperfect but capable national interoperability ecosystem existed, made up of the fabric of provider EHRs and payor databases that work in cooperation with not-for-profit and state-run HIEs/HDUs. That ecosystem continues to evolve and advance. While robust, statewide HIEs do not exist in every state, that is somewhat due to past federal policies diverting resources and attention to non-state-based approaches, including TEFCA.

National exchanges, such as TEFCA, do not offer enhanced identity management or security compared to their state-based counterparts. State-based HIEs and HDUs have been recognized as having a high degree of trust with their participating organizations, in part because of the multi-stakeholder governance of the HIE/HDU and the relative "localness" of relationships.

Furthermore, HIEs and HDUs must match and manage patient/citizen identities across all participants in order to perform their core value-creating functions. The burden of identity management and interoperability on the healthcare and public health systems (as well as other state agencies including Medicaid) could be lessened by leveraging the existing identity management capabilities of statewide HIEs and HDUs. A demonstration of this approach can be seen in Colorado.

The only unique aspect of TEFCA is the endorsement and direction-setting from the Federal Government. While imperfect and incomplete, the national interoperability ecosystem that was evolving prior to TEFCA was capable and accelerating. The announcement of TEFCA in 2016 somewhat diverted attention and resources for innovation away from not-for-profit and private sector networks and slowed the development of new interoperability functions using the existing ecosystem and HIE/HDU infrastructure.

We propose to CMS and ONC that:

- A nationwide data exchange strategy should be less prescriptive and burdensome by making use of existing intra and inter state HIE/HDU infrastructure.
- A nationwide data exchange strategy should include collaboration with HIEs and HDUs to enhance data quality and availability.
- Resources be better distributed among state-based and state-run HIEs and HDUs and national exchanges to ensure continued innovation across the data exchange ecosystem.